

Clinical Findings

Cutaneous Lesions

Molluscum contagiosum (MC) typically presents as small, pink, pearly, or flesh-colored papules that gradually enlarge, occasionally reaching up to 3 cm in size—referred to as "giant molluscum." As the lesions grow, they often develop a characteristic dome-shaped, opalescent appearance with a central depression or umbilication (Fig. 195-12A). A white, curd-like material can be expressed from this central core.

Most individuals develop multiple papules, commonly located in intertriginous areas such as the axillae, popliteal fossae, and groin. Genital and perianal lesions may occur in children and are rarely indicative of sexual transmission in this age group.

Lesions may appear in clusters or in a linear arrangement, the latter often due to koebnerization—development of new lesions at sites of skin trauma. Perilesional erythema and eczematous changes, known as molluscum dermatitis, can occur. Erythematous lesions (see Fig. 195-12B) are thought to represent an immune response to the virus.

In patients with advanced HIV infection or acquired immunodeficiency syndrome, molluscum lesions may be extensive, large, and widespread, affecting both genital and extragenital sites.

Special Tests

Diagnosis is usually clinical, but confirmation can be obtained when needed by examining the central core material. A crush preparation of the lesion contents followed by Giemsa staining may reveal characteristic findings (Fig. 195-13).

Histopathologic examination shows epidermal hyperplasia and hypertrophy. Above the basal layer, enlarged keratinocytes contain large intracytoplasmic inclusions known as Henderson-Paterson bodies (Fig. 195-14). These inclusions increase in size as infected cells migrate toward the stratum corneum.

Differential Diagnosis

Consider the following in the differential diagnosis: - **Verrucae** (common warts) - **Pyogenic granulomas** - **Amelanotic melanoma** - **Basal cell carcinomas** - **Appendageal tumors**

In immunocompromised patients, fungal infections such as **cryptococcosis**, **histoplasmosis**, and **penicilliosis** should also be considered.

Complications

While many patients are asymptomatic, complications may include: - **Pruritus**, particularly problematic in individuals with underlying atopic dermatitis - **Chronic conjunctivitis** and **punctate keratitis** in cases involving eyelid lesions - **Secondary bacterial infection**, especially when lesions are scratched or excoriated